

Advise the patient with DS-TB or INH mono-resistant TB

Health for All

78

- Provide TB counselling and refer for community or workplace adherence support.
- Educate about TB treatment side effects 78 and advise to return promptly should they occur.
- Educate about infection control: adequate ventilation/open windows (if area is not well ventilated, to wear a face mask), cough/sneeze into upper sleeve or elbow. Wash hands with soap regularly.
- If patient smear positive, advise to stay home from work for the first 2 weeks of treatment.
- Alert to the risks of smoking 141 and alcohol/drugs and support patient to change 177. If patient chooses to continue, advise safe alcohol use 142 and to continue taking TB medication daily.
- Give **enhanced adherence support** to the patient with poor adherence 173:
 - Educate on the importance of adherence and the risks of resistance.
 - Ask about alcohol/drug use 142, stress/anxiety/depression 86 and side effects 97.
 - Refer for support: adherence counsellor, support group, treatment partner, community health worker.

Treat the patient with drug-sensitive or INH mono-resistant TB

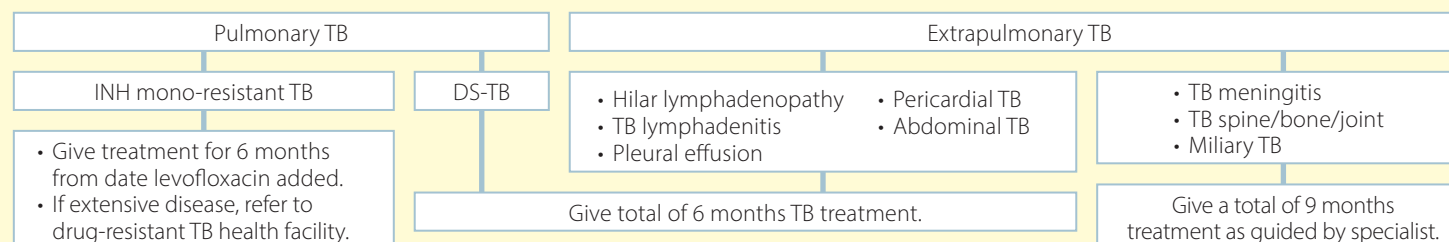
• If drug-sensitive TB (DS-TB):

- Treat the patient (whether a new or retreatment case) 7 days a week for 6 months:
 - Give intensive phase **rifampicin/isoniazid/pyrazinamide/ethambutol (RHZE)** for 2 months. Prolong for 1 month if 7 week smear positive 97.
 - Then, if clinically improving (and 7 week smear negative if done), change to continuation phase **rifampicin/isoniazid (RH)** for a further 4 months.
 - If TB meningitis, TB bones/joints or miliary TB, extend treatment to 9 months (2 months RHZE/7 months RH) or as guided by a specialist.
- If patient pregnant, misuses alcohol, or has diabetes, epilepsy or BMI < 18.5: also give **pyridoxine** 25mg daily. Stop on completion of TB treatment.

• If INH mono-resistant TB:

- Give daily 7 days a week: **rifampicin** 10mg/kg + **pyrazinamide** 25mg/kg + **ethambutol** 15mg/kg + **levofloxacin** 750-100mg, until TB treatment completed. To ensure adherence, give fixed dose combination tablets **RHZE** even though isoniazid (H) may not be active and add **levofloxacin** (see table).
- If inhA mutation only, consider giving high-dose **isoniazid** (up to total of 10mg/kg/day in total). If unsure, present to NCAC¹.
- Give **pyridoxine** 50mg daily until TB treatment completed.

• Decide treatment duration:



- **If HIV positive:** if on ART, adjust medication/dosing 97. If not on ART 114.
- Check that patient is up to date with his/her COVID-19 vaccination.

Dose according to weight and adjust as weight increases

	RHZE (150/75/400/275mg)
25-37kg	2 tablets
38-54kg	3 tablets
55-70kg	4 tablets
≥ 71kg	5 tablets

	RH
25-37kg	2 tablets (150/75mg)
38-54kg	3 tablets (150/75mg)
≥ 55kg	2 tablets (300/150mg)

	Levofloxacin
< 33kg	15-20mg/kg
33-50kg	750mg
≥ 51kg	1000mg

Review the patient with drug-sensitive or INH mono-resistant TB

- If stable, review monthly during intensive phase, then arrange for treatment collection through RPCs². Avoid treatment collection through RPCs if patient has TB and HIV.
- Review again at week 23.
- Advise to return sooner if worsening or side effects develop.
- Assess patient for TB preventive treatment (TPT) 89 and check for TB yearly for 2 years after completing TB treatment 92.

¹National Clinical Advisory Committee. ²RPCs - repeat prescription collection strategies make it easier and quicker for patient to collect their chronic medications and include 'facility pick-up points' (FAC-PUPs), 'external pick-up points' (EX-PUPs) and clubs. Medications are pre-dispensed by Central Dispensing Unit (CDU) or Central Chronic Medicine Dispensing and Delivery (CCMDD).